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The
**Sleep
Training**

PLAYBOOK



A research-backed plan for exhausted parents who are done winging it

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Dad of Two

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~ 60 minute read

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A note on medical advice: This book is based on published pediatric research and the author's experience applying it. It is not a substitute for guidance from your child's pediatrician. Every baby is different. If something in this book does not match what you are seeing, trust your doctor over this guidebook.

CHAPTER 1

The Remote in the Fridge

FIRST SON & SECOND SON

The remote control was in the fridge. I found it there at 3am, next to the leftovers. I had put it there myself, probably while feeding First Son, with no memory of doing it. That is where we were. Sleep deprived to the point of rearranging household objects without realizing it.

First Son did not sleep. Not consistently. Not in the way the books described. We had tried everything. Feeding to sleep. Rocking to sleep. The yoga ball. The white noise machine. Co-sleeping, which helped some, until it did not. At five months, we started graduated extinction. Put him down awake. Left the room. Came back at intervals. Did not pick him up. It worked in a week. It worked so well that I spent two years asking myself why we did not do it sooner, and the answer is always the same: we did not understand what we were dealing with.

This book is what I wish I had at four months. Not a method recommendation. The full picture of how infant sleep works, what the research actually supports, and how to choose. With Second Son, I went in prepared. The plan held because I understood it.

THE SCIENCE

The research on infant sleep training is more robust than most parenting debates would suggest. A 2006 meta-analysis by Mindell reviewed 52 studies on behavioral sleep interventions. Ninety-four percent found them effective. A 2016 randomized trial by Gradisar found that the method helped babies fall asleep faster, lowered cortisol, and produced no differences in attachment at twelve months. A 2012 study by Price followed 326 families for five years and found zero differences in emotional health, behavior, or parent-child closeness.

THE LESSON

Sleep deprivation is not a parenting rite of passage you have to earn. There is an evidence-based path out of it. You just need the full picture before you start.

CHAPTER 2

Nothing Is Wrong With Your Baby

FIRST SON & SECOND SON

Every night for four months, I lay awake wondering what was wrong with First Son. He was bright, healthy, growing. But he woke every 45 minutes. He could not fall back asleep without being fed or rocked. Nothing was wrong with him. Nothing was wrong with us. He had a sleep association, and I had built it.

THE SCIENCE

Sleep associations are the conditions present when a baby falls asleep. If your baby is fed, rocked, or held to sleep, those are the conditions their brain associates with sleeping. When they wake at the end of a sleep cycle -- which every human does -- they need those same conditions to return to sleep. If the conditions are gone, they signal. That is not a sleep problem. That is a learned association.

The goal of sleep training is not to make the baby stop needing comfort. It is to give the baby the skill of falling asleep in conditions that will still be present at 2am: the crib, the dark, the white noise, the baby's own body. Once those become the association, independent sleep follows.

THE LESSON

Nothing is wrong with your baby. You built a sleep association. Sleep training replaces it with a more sustainable one. That is all this is.

TRY THIS

1. Write down the exact conditions present when your baby falls asleep right now. Those are the associations you are working with.
 2. Identify which associations require your active involvement at 2am. Those are the ones that need to change.
 3. Accept that the method does not create a new problem. It solves the existing one.
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CHAPTER 3

Safe Sleep First

THE ABCS

Safe sleep is the baseline everything else is built on. The American Academy of Pediatrics is clear: until twelve months, babies should sleep Alone, on their Backs, in a Crib. On a firm, flat surface. No loose bedding, no pillows, no bumpers, no inclined sleepers.

SIDS risk is highest from one to four months. The back sleeping position reduces SIDS risk by 50 percent. Pacifiers during sleep reduce SIDS risk and are encouraged by the AAP. Swaddling helps in the first two months -- stop as soon as the baby shows signs of rolling. Room temperature: 68-72 degrees F (20-22 C). Tobacco smoke exposure is an independent SIDS risk factor even when smoking occurs outside.

THE LESSON

Safe sleep is not optional and it is not negotiable. Get this right before anything else.

CHAPTER 4

How Baby Sleep Actually Works

THE BIOLOGY

A newborn's sleep is chaotic because it is supposed to be. The circadian rhythm is not yet functioning. It develops in response to light exposure and feeding patterns over the first few months.

The four-month sleep regression is actually a maturation. At around four months, a baby's sleep architecture permanently shifts from two stages to four, matching adult sleep structure. This is why a baby sleeping three-hour stretches at eight weeks suddenly wakes every 45 minutes at four months. It does not go back. It is the signal that sleep training is now possible.

Sleep cycles in infants run about 45 minutes. An infant with an independent sleep skill returns to sleep on their own at the end of each cycle. An infant with a sleep association signals for the association to be replaced.

Wake windows by age (ranges, not rules): 0-6 weeks: 45-60 min. 2-3 months: 60-90 min. 4-5 months: 90 min-2 hrs. 6-8 months: 2-2.5 hrs. 9-12 months: 2.5-3.5 hrs. 12-18 months: 3.5-4.5 hrs. Watch the baby, not the clock.

THE LESSON

The four-month regression is not regression. It is readiness. The biology is on your side once you know how it works.

Choosing Your Method

FIRST SON & SECOND SON

With First Son, I did not choose a method. I had a vague sense that I was supposed to let him cry or not let him cry, and I toggled between the two depending on how tired I was and how guilty I felt. Some nights I rocked him for an hour. Other nights I lasted four minutes. The inconsistency was the problem. He was getting a different signal every night and had no way to learn from it because the rules kept changing.

With Second Son, I picked a method before night one. I understood what it was, why it worked, and what the first week would look like. When night two was harder than night one, I did not panic. When my wife wanted to go in sooner, we had already agreed on the intervals. The plan held because we had made the decision in advance, not in the moment.

THE METHODS

Graduated extinction -- "extinction" is behavioral science terminology for letting the crying reduce over time by not reinforcing it, the Ferber method by another name -- is the approach with the strongest evidence. Put the baby down awake. Leave. Return at set intervals (3 min, 5 min, 10 min) for brief check-ins without pick-up. Resolution typically takes five to seven nights.

The chair method places a parent next to the crib with verbal comfort but no pick-up. Every few nights, the chair moves farther from the crib. Takes one to two weeks. For some babies, a visible but unreachable parent is more frustrating than being alone.

Bedtime fading pushes bedtime later until the baby falls asleep quickly, then gradually moves it earlier. Minimal crying. Takes two to six weeks. Before starting any method: baby is not actively ill, no untreated reflux, no vaccination in the last 24-48 hours, no signs of airway obstruction.

THE LESSON

The method you can follow consistently is the right method. A perfect plan abandoned on night two is worse than an imperfect plan executed all the way through.

TRY THIS

1. Be honest about your tolerance. If you will go in after fifteen minutes regardless, do not start with graduated extinction.
2. Make the decision before night one. Couples who disagree during training cause more problems than they solve.

3. Commit to seven nights before drawing any conclusions.

Game Night

FIRST SON & SECOND SON

With First Son, night one I went in after four minutes. I told myself it was a check-in. I picked him up. He stopped crying immediately. I put him back down. He cried harder. I went in after two minutes. By night four, nothing had changed and I had decided sleep training was cruel and did not work. Both conclusions were wrong.

With Second Son, I had read the Gradisar trial. I knew night two would probably be harder than night one. I had told my wife. We had agreed on the intervals. Night one: 47 minutes of crying, then sleep. Night two: 62 minutes. Night three: 28 minutes. Night four: 9 minutes. Night five: 4 minutes. Night six: nothing. He put himself to sleep.

THE PROTOCOL

Peak crying on night one or two, rapid decline after. Night two is worse because the learned behavior is being tested before it extinguishes. This is not failure. It is the process.

Consistency is not optional. If you pick the baby up after twenty minutes on night one, you have taught them that twenty minutes of crying produces results. If you break the plan, put them back down once calm but still awake, resume intervals, and continue. One slip sets you back one night, not seven.

Night feeds: most five- to six-month-olds still need one. Keep it dim, no talking, no eye contact. Feed, burp, crib, out.

THE LESSON

Night two is supposed to be worse. When it is, you are on track. When it is not, you are also on track.

TRY THIS

1. Write "night two is supposed to be worse" somewhere visible before night one.
 2. Set intervals in advance: 3 min, 5 min, 10 min, repeat at 10. Stick to them.
 3. The parent not doing check-ins should not watch the monitor.
 4. Commit to seven nights before drawing any conclusions.
 5. If you pick them up, put them back down once calm but awake and continue.
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The Team

FIRST SON & SECOND SON

The worst argument my wife and I had in the first year of First Son's life was not about money or time or whose turn it was to change a diaper. It was about sleep. She wanted to go in. I wanted to wait. Or the opposite. It changed night to night because we had no plan, so every decision was made in real time by two exhausted people who were scared and blaming each other for something neither of us knew how to fix.

With Second Son, we had the argument before night one. Kitchen table, nap time, no one crying. She told me she was not sure she could listen to him cry for more than twenty minutes. I said the research suggested it could go longer. We agreed on a cap: if he was still crying hard after an hour, we would both go in, soothe him, and try again the next night. We never hit the cap. Having it there gave her permission to trust the process. That conversation took twenty minutes. It saved us weeks.

THE SCIENCE

The variable most likely to predict whether sleep training succeeds is parental consensus. Two parents who disagree about the plan will break on different nights and for different reasons -- teaching the baby that sustained escalation eventually works.

Mothers and fathers typically do not experience sleep training nights the same way. The nursing state creates a different relationship between a mother's nervous system and the baby's distress signal. For many families, the non-nursing parent taking the lead on check-ins is the only way the nursing parent survives night one without breaking.

THE LESSON

Training a baby to sleep is a team sport. The plan needs to be made together, before the first night, when neither partner is listening to a crying baby.

TRY THIS

1. Have the planning conversation at least three days before you start. Not during a hard night.
2. Assign the check-ins to one parent in advance.
3. Give the non-check-in parent a specific job: timing the intervals, tracking the night, making tea.

4. Prepare a script for people who push back: "We looked at the research and made a decision. We need one week of consistency."
 5. If your partner is absent or unwilling, you can do this alone. You need their non-interference, not their active help.
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Fed Is Best

FIRST SON & SECOND SON

My wife breastfed both boys. With First Son, the last thing she did every night was nurse him to sleep. It worked for three months. Then it was the only way he would go down, and she became the sole bedtime parent and the sole 1am parent and the sole 4am parent. I felt useless. She felt trapped. We were both right.

With Second Son, we moved the last feed to the start of the bedtime routine at six weeks. She nursed him in the living room with the lights on. Then I took over: bath, pajamas, books, crib. He learned to fall asleep in the crib, not at the breast. When we started training at five months, the transition was smoother because the strongest sleep association had already been separated from the act of falling asleep.

THE SCIENCE

Sleep training and feeding are two separate systems. You can train a well-fed baby. You cannot train a hungry baby. Most babies can consolidate one extended stretch by three to four months. By nine months, a healthy growing baby on solid foods typically does not need overnight calories.

Breastfeeding and sleep training are compatible. Many breastfeeding families train successfully by having the non-nursing parent lead overnight check-ins. The nursing parent feeds once per night at a set time; all other wakings are handled by the plan.

THE LESSON

Train the sleep. Feed the baby. These are not the same decision.

Naps

FIRST SON & SECOND SON

Nighttime training with Second Son took one week. Nap training took three. I was not prepared for that. I assumed that once he learned to fall asleep independently at night, naps would follow automatically. They did not. He went down beautifully at 7pm and then fought his 9am nap like it had personally wronged him.

With First Son, naps were never trained at all. They happened in the car, in the stroller, in someone's arms, or not at all. Whatever worked in the moment, which meant it was never the same twice. What I did not understand with either son until I did the reading: naps and nighttime are different biological systems. Nighttime has melatonin, darkness, and hours of accumulated sleep pressure working in its favor. Naps have none of that. They take longer to train and require their own strategy.

THE SCIENCE

Nap training typically follows one to two weeks after successful nighttime training. Use the same method, the same room, the same routine. Nap transitions are among the most disruptive points in the first two years. The two-to-one transition occurs between 15 and 18 months; the one-to-zero between two and a half and three and a half years. Transition periods last two to six weeks and are messy -- some days one nap works, some days two are needed.

THE LESSON

Fix nights first. Naps follow. Use the same method in the same room with the same routine.

Co-Sleeping

THE EVIDENCE

Co-sleeping is practiced globally and the research on it is genuinely complex. Bed-sharing on a standard adult mattress with non-smoking parents who have not consumed alcohol or sedating medication carries meaningfully lower risk than bed-sharing with any of those factors present. A 2013 meta-analysis by Carpenter found an adjusted odds ratio of 2.7 for SIDS with any bed-sharing versus crib sleeping. There is genuine uncertainty here, not reassurance.

If you co-sleep: firm mattress, no pillows or loose bedding near the baby, smoke-free household, no alcohol or sedating medication for either parent, baby on their back between the mattress edge and one parent -- not between two adults.

A firm-sided bedside bassinet attached to the adult bed provides the proximity benefits of co-sleeping at significantly lower risk. This is the AAP-endorsed approach for parents who want closeness.

THE LESSON

If you co-sleep, do it as safely as possible. The risk is not binary. The environment matters.

Regressions

FIRST SON & SECOND SON

At ten months, Second Son stopped sleeping through the night. Three weeks earlier, everything was perfect. Down at 7, asleep in minutes, up at 6:30. Then, without warning, he started waking at midnight, standing in the crib, screaming when I left the room. My first thought was that sleep training had failed. It had not. He was going through the eight-to-ten-month developmental surge. He had just started pulling to stand. Separation anxiety had kicked in. His brain was busy, and the busyness was spilling into the night.

With First Son, I would have responded by going back to whatever stopped the crying: rocking, feeding, holding. With Second Son, I held the line. Same routine, same response, check-ins at intervals, no new associations. It lasted eleven days. Then it was over, and he slept through the night as if nothing had happened. The skill was still there. It just needed to wait out the storm.

THE SCIENCE

Sleep regressions are temporary disruptions that correspond to developmental leaps, illness, or environmental change. The skill is not lost -- it is temporarily overridden. Common regression windows: 4 months, 8-10 months, 12 months, 18 months, 2 years. Each typically lasts two to six weeks. A well-trained baby returns to baseline faster than an untrained one.

Night terrors are distinct from regressions. They are NREM arousal parasomnias, occurring in deep non-REM sleep in the first third of the night, in children aged two to six. The child screams and does not recognize the parent. Do not try to wake or console. Stand nearby, ensure physical safety, and wait. It passes in five to fifteen minutes with no memory in the morning.

THE LESSON

Regressions end. Avoid rebuilding the associations you removed. Hold the line and return to the plan when the disruption passes.

When Training Does Not Work

WHEN TO STOP AND CHECK

Most sleep training challenges are method issues, not medical ones. Inconsistency, starting too early, and misread wake windows account for the majority of cases where training does not progress as expected.

Stop and speak to your pediatrician if: the baby has chronic snoring or labored breathing during sleep; significant untreated reflux; crying escalates consistently after five nights of consistent effort; or you observe unusual patterns in breathing, color, or responsiveness.

A baby who was sleeping well and suddenly is not, without a clear developmental trigger, warrants a medical check. Ear infections and illness manifest as sleep disruption. Treat the cause before returning to training.

THE LESSON

If training is not working after five consistent nights, check wake windows, environment, and consistency. If those are right, check for a medical cause.

After the Game

FIRST SON & SECOND SON

When Second Son started sleeping through the night, I did not know what to do with the evenings. That sounds absurd. But for months, my nights had been structured around feeding, rocking, checking the monitor, and lying awake wondering if the next waking was five minutes or five hours away.

Suddenly there were three hours between his bedtime and mine. My wife and I sat on the couch and looked at each other. She said: what do we do now? I said: I have no idea. We watched a movie. The whole thing, without pausing. It felt like a vacation. Sleep training gave us that. But it also gave us something harder to measure: we stopped dreading bedtime, my wife stopped carrying the nights alone, and we started being people again -- not just parents in survival mode.

WHAT DONE LOOKS LIKE

Done looks like a baby who is put down awake and asleep within twenty minutes without sustained crying. It looks like waking once per night or not at all. It looks like naps that happen at predictable times. It looks like the baby not requiring your active presence to fall asleep.

There is no ceremony. One day you notice that the whole process has become unremarkable. That is what done looks like. Sleep is a skill, not a permanent achievement. A trained baby recovers from any disruption in two to four nights. Return to the plan without drama and the skill reasserts itself.

THE LESSON

Done is a pattern, not a moment. When the independent settling is consistent and the wakings have stopped, the skill belongs to them now.

TRY THIS

1. Keep the bedtime routine consistent after training is complete. The routine is the skill's anchor.
 2. When a regression hits, return to the method. Two to four nights restores the baseline.
 3. Do not rebuild associations to get through a hard night.
 4. Give yourself credit. You did the research, you held the plan, you got through the hard nights. That is the job.
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APPENDIX A

Sleep Plan Worksheet

BEFORE YOU START

Baby's age: ____ Current weight: ____ Feeding method: ____

Method chosen (circle): Graduated extinction / Chair method / Bedtime fading

Bedtime target: ____ Wake window before bedtime: ____ Night feeds to keep: ____

CHECK-IN INTERVALS (GRADUATED EXTINCTION)

Night 1: 3 min / 5 min / 10 min (repeat at 10). Night 2: 5 min / 7 min / 10 min. Night 3+: 5 min / 10 min / 12 min.

SLEEP ENVIRONMENT CHECKLIST

1. Firm, flat surface. No loose bedding, pillows, bumpers.
 2. Room temperature 68-72 F (20-22 C).
 3. Dark room. Blackout curtains if possible.
 4. White noise at 65-70 dB, placed away from baby.
 5. Smoke-free household.
 6. Baby on back.
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APPENDIX B

Wake Windows and Schedules

WAKE WINDOWS BY AGE

0-6 weeks: 45-60 min. 2-3 months: 60-90 min. 4-5 months: 1.5-2 hrs. 6-8 months: 2-2.5 hrs. 9-12 months: 2.5-3.5 hrs. 12-18 months: 3.5-4.5 hrs.

SAMPLE 6-MONTH SCHEDULE

7:00am Wake. 9:00am Nap 1 (45-60 min). 12:30pm Nap 2 (1.5-2 hrs). 7:00pm Bedtime routine. 7:30pm Down awake. One night feed if needed.

APPENDIX C

Recommended Resources

RESEARCH

Gradisar et al. (2016). "Behavioral Interventions for Infant Sleep Problems." *Pediatrics*. The most rigorous recent RCT on graduated extinction and bedtime fading.

Mindell et al. (2006). "Behavioral Treatment of Bedtime Problems and Night Wakings in Infants and Young Children." *Sleep*. Meta-analysis of 52 studies.

Price et al. (2012). "Tolerability, Blinding, and Safety of Behavioral Sleep Interventions." *Pediatrics*. Five-year follow-up of 326 families. No long-term harm.

BOOKS AND COMMUNITY

Solve Your Child's Sleep Problems by Richard Ferber. *Precious Little Sleep* by Alexis Dubner. Reddit r/sleeptrain (most active community). Facebook: Taking Care Babies Support Group.

APPENDIX D

Is This Normal?

COMMON QUESTIONS AT 2AM

My six-week-old wakes every two hours. Normal. Circadian rhythm not yet developed. Wait it out.

My four-month-old was sleeping well and now wakes every 45 minutes. Normal. The four-month maturation. Begin sleep training.

My baby falls asleep easily but wakes multiple times. Sleep associations. Chapter 2 explains. Chapter 6 fixes.

My baby cried five minutes then fell asleep without a check-in. Normal. The training worked.

Night six and it is getting worse. Check for illness, then consistency. If both clear, speak to your pediatrician.

My toddler screams at night and does not recognize me. Possible night terror. Chapter 11. Do not wake or console. Stand nearby and wait.

ABOUT THE AUTHOR

Jack Hartley is a father of two boys and the author of *The Potty Training Playbook*. He is not a doctor, a sleep consultant, or a child psychologist. He is a parent who went looking for a single resource that pulled together the science of infant sleep in plain language, could not find one, and wrote it himself. Between the two playbooks, he reviewed more than forty peer-reviewed studies and clinical guidelines from the American Academy of Pediatrics. He is a parent who did the reading so you don't have to.

The Sleep Training Playbook is part of the FamilyForce series of research-backed guides to the milestones that matter.

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